CTEP Concept Submission: Perioperative Daraxonrasib (RMC-6236) in Resectable KRAS-Mutated Pancreatic Ductal Adenocarcinoma

Design: Open label, single arm, 3+3 **Phase:** 1, first in human	**Population:** Resectable or borderline resectable KRAS G12-mutated PDAC
Enrollment: Up to 18 treated participants **Sites:** One, agreement pending	**Co-primary:** DLT and MTD or RP2D; 30-day device- or procedure-related SAEs

CEO Kevin Kawchak, ChemicalQDevice | kevink@chemicalqdevice.com | 0009-0007-5457-8667
San Diego, California | August 6, 2026 | Intended site: UC San Diego Moores Cancer Center (feasibility stage)

Submitted as an independent scientist under the funding approach set out in Science: A New Golden Age *(July 2026). Not medical or regulatory advice, and not endorsed by the FDA, NIH, NCI, HHS, an IRB, ICH, or any sponsor. UC San Diego is named as the intended partner of choice and is not a sponsor, site, or endorser.*

1 Why an Unaffiliated Investigator Is Submitting a Concept

Science: A New Golden Age uses this disease area as its own example of scientific triumph: "Within living memory, cancer has been transformed from a death sentence to a treatable condition for millions of Americans" [1]. Pancreatic ductal adenocarcinoma is where that sentence is least true; five-year survival remains in the low teens [2]. The report asks the roughly **$200 billion** annual federal research portfolio to invest directly in individual researchers rather than deferring to incumbents, and its annex asks agencies to prioritise foundational work in the biological sciences over the broader life-sciences category [3].

This concept is submitted on that basis by an unaffiliated investigator who already holds the documents a concept review normally asks a cooperative group to produce: a published Phase 1 protocol, a Phase 2 protocol, an Investigational New Drug application, and a fixed statistical and safety plan [4, 5, 6]. What is missing is a network, a qualified site, and CTEP's own review - as illustrated in Table 1 and Figure 1.

Usual concept-stage gap	Status here
Protocol not yet written	Published, with a DOI, before submission
Statistical plan indicative	Fixed: 3+3 escalation, exact intervals, treated participants, safety summary
Correlative plan unfunded	Specimen-level pharmacodynamic plan costed inside the budget
Investigator network in place	Absent. One site is sought and none is agreed

Table 1. Four things a concept review usually has to establish, and where each stands here. The last row is the one genuine gap, and it is the reason for the submission.

2 Pharmacologic Case

Daraxonrasib (RMC-6236) is a RAS(ON) multi-selective inhibitor. RASolute 302 reported median overall survival of **13.2 months** against **6.6** for chemotherapy in the RAS G12 population of previously treated metastatic pancreatic cancer seen in Table 2 [7]. A ten-arm quantitative systems pharmacology simulation with 250 ordinary differential equations per patient had returned **12.8** against **5.4 months** nine months earlier [8]. That proximity is chronology, not validation: 1000 simulated against 241 enrolled, a combination against a single agent, KRAS G12C against a primarily G12D and G12V population.

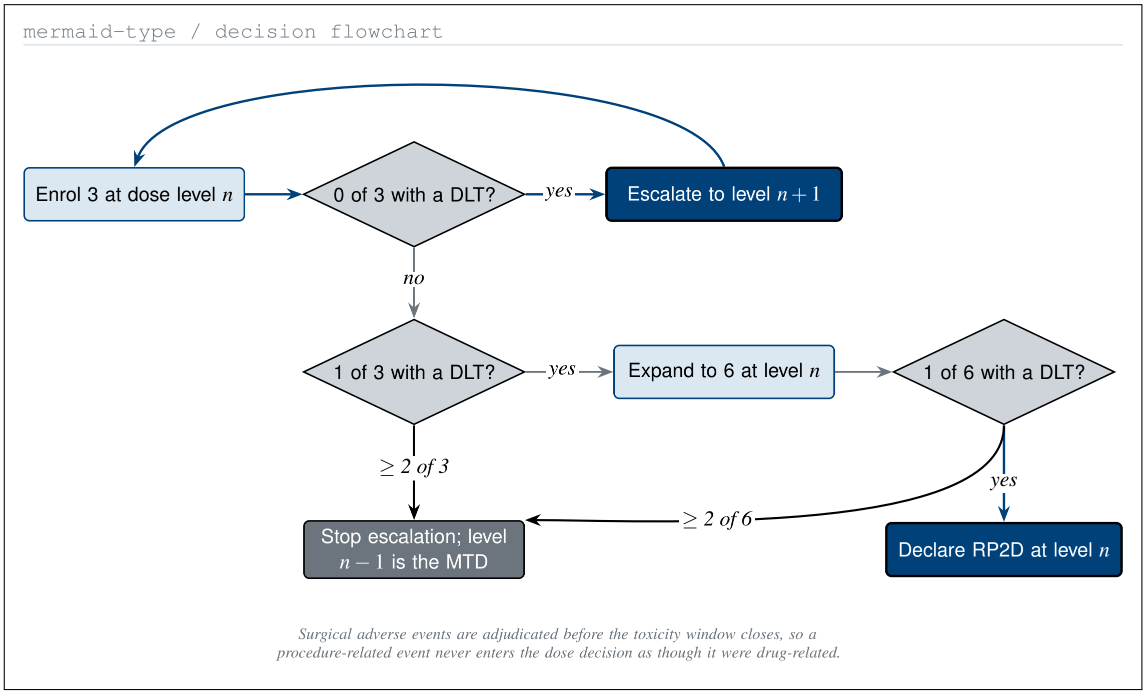

Figure 1. The 3+3 rule as it will be executed, including the return edge to the next cohort. The adjudication note is the part that distinguishes this from a standard escalation in a non-surgical population.

Source and date	Test arm	Control	Reading and limitation
QSP simulation, Aug 2025	12.8 mo mOS	5.4 mo mOS	Best arm HR 0.25; assumes no acquired resistance and ideal pharmacodynamics
Digital twin, Oct 2025	12.1 mo mOS	not applicable	Best mPFS HR 0.31; no patient-specific PK or tumour growth parameters
RASolute 302, May 2026	13.2 mo mOS	6.6 mo mOS	Metastatic, previously treated; says nothing about the resectable setting

Table 2. Each source with its stated limitation in the same row, so no line of the table can be read without the caveat that qualifies it. Limitations are the authors' own, not added here.

Neither simulation nor the metastatic readout establishes anything about perioperative exposure in resectable disease, which is what this concept proposes to measure directly in the resection specimen.

Positioning note. *First in human refers to the integrated surgical and advisory workflow, not to daraxonrasib, which is investigational and already in Phase 3 evaluation. The robotic configuration is specified by manufacturer, model, cleared configuration, arm count, and software version at the site agreement, and no drug supply or letter of authorization is in place with the agent's developer.*

3 Adjudication and Safety Reporting

A combined drug and device study in a surgical population has one structural hazard that a standard escalation does not: an adverse event can be attributed to the drug (Figure 2), to the operation, or to the advisory system, and the dose decision depends on which (Figure 3). Attribution is therefore adjudicated by a blinded reviewer before the toxicity window closes.

Reporting follows attribution. An event adjudicated as drug-related enters the IND safety report; one adjudicated as device- or procedure-related enters the device reporting path and the feasibility count; one adjudicated as advisory-related enters both the device path and the advisory audit.

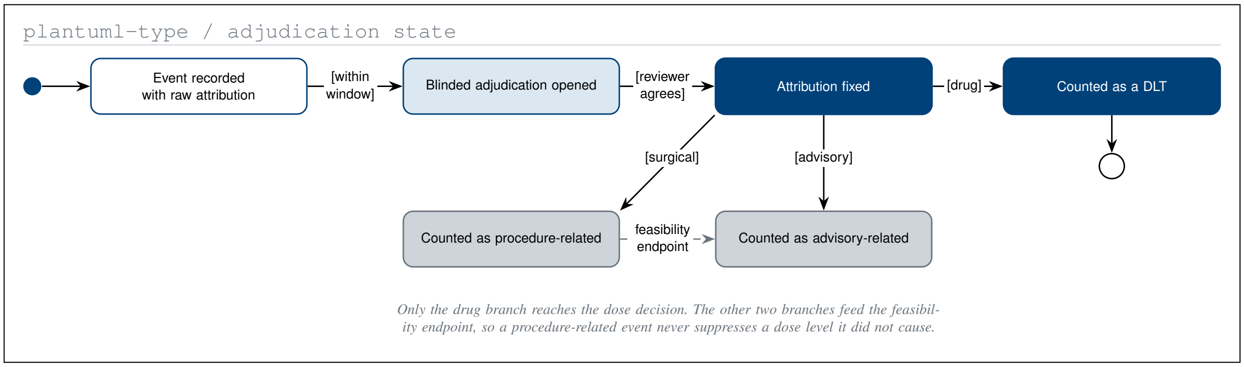

Figure 2. Adverse-event adjudication as a guarded state machine. Only the drug branch reaches the dose decision; the surgical and advisory branches feed the feasibility endpoint instead.

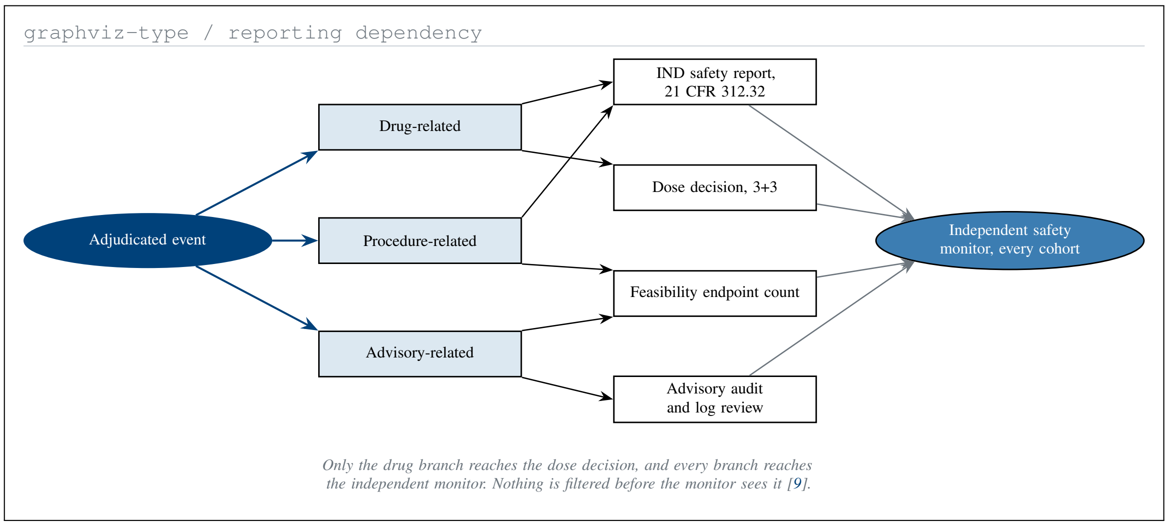

Figure 3. Where an event goes once its attribution is fixed. Every branch reaches the independent safety monitor, and only the drug branch reaches the dose decision.

d2-type / schedule of assessments grid

Assessment	Day −14	Day −1	Day 0 resection	Day +30	Day +90
Plasma pharmacokinetics	✓	✓	✓		
Tumour pathway PD			✓		
ctDNA clearance	✓			✓	✓
Pathologic response			✓		
Adverse events, CTCAE	✓	✓	✓	✓	✓

Figure 4. The schedule of assessments. Only two assessments are taken at the resection itself, which is what makes specimen handling the single hardest operational item in the study.

4 Schedule, Budget, and the Partner Site

The request is **$700,000 per year for five years**, $3,500,000 total, with the correlative pharmacodynamic work costed inside it rather than deferred to a supplement [10]. Moores Cancer Center is the intended partner of choice, approached at the feasibility stage only. UC San Diego is not a partner, sponsor, site, or endorser, because no written authorization exists.

Scope of Claims

Claimed: a published protocol, a published IND, a fixed statistical and safety plan, and an adjudication rule under which a procedure-related event cannot suppress a dose level it did not cause, depicted in Figure 4. Not claimed: that the simulations validate RASolute 302, that the advisory system holds any regulatory clearance, that anything here has been used in a human, or that UC San Diego has agreed to anything. The trial does not exist until a site, an IRB, active IND, monitoring, insurance, and sponsor of record are in place.

Availability and Conflicts

Every work cited to a DOI is openly deposited; code, verification notebooks, and figure sources are in the repository [11]. The applicant holds no equity in Revolution Medicines or any robotic surgery vendor, and no sponsor, contract research organization, or medical society reviewed this document.

References

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